

Clinical Progress Note

Care360 Health System | Confidential Clinical Record

Patient ID: P1007

Date: 2026-05-14

Section 1: Chief Complaint

Patient P1007 presents for a scheduled follow-up of type 2 diabetes mellitus and essential hypertension. Reports intermittent morning fatigue and mild bilateral lower-extremity swelling over the past two weeks.

Section 2: History of Present Illness

62-year-old established patient with a five-year history of type 2 diabetes (ICD-10 E11.9) and hypertension (ICD-10 I10). Home glucose logs show fasting values between 140 and 170 mg/dL. Reports adherence to metformin but occasional missed doses of lisinopril. Denies chest pain, dyspnea at rest, or visual changes.

Section 3: Objective / Vitals

BP 148/92 mmHg, HR 78 bpm, Temp 98.4 F, RR 16, SpO2 98% on room air. Weight 91.2 kg, BMI 31.4. Trace pedal edema bilaterally. Cardiac and pulmonary exam unremarkable.

Section 4: Assessment

1) Type 2 diabetes mellitus, suboptimally controlled - most recent HbA1c 8.1%. 2) Essential hypertension, above goal. 3) Mild peripheral edema, likely related to blood pressure control.

Section 5: Plan

Increase metformin to 1000 mg twice daily. Reinforce lisinopril adherence and increase to 20 mg daily. Order fasting lipid panel and basic metabolic panel. Diabetic foot exam performed. Follow up in eight weeks with repeat HbA1c. Patient counseled on diet and daily home BP monitoring.